



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : زينب محمد مرسى حسن
المحافظة : القاهرة
الحى / المدينة : الزيتون
رقم البطاقة : 25403240103429 رقم التليفون : 01013433693
العمر (تاريخ الميلاد) : 24-03-1954 المهنة : ربه منزل
العنوان : -----
تاريخ تسجيل الإستمارة : 10-10-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
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التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- / د. / ----- / د. / ----- / د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----
قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: زينب محمد مرسي حسن National ID: 25403240103429	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الناحية

زينب
محمد مرسى حسن
٢ ش. مصطفى بك
الزيتون - القاهرة

٢٥٤.٣٢٤.٠١.٣٤ ٢٩



LM3088202



٢.٢٥/١١ ٢٥٤.٣٢٤.٠١.٣٤ ٢٩
ربة منزل
النش
مسلمة
متزوجة
نشاط عبدالوهاب تالغ احمد فايد
البطاقة سارية حتى ٢٠٣٢/١١/٣١



Patient Name : الأستاذة / رانيا محمد مرسي
Patient No. : 25403240103429
Path No. : 2024/24
Sex : Female
Age : 170 Y
Referring Doc :

Request No. : 13005748
Request Date : 12/06/2024
Verification Date : 01/07/2024
Financial Category : أورام دار السلام - تلكة الدولة
Department : Out Patient
Registered by : I.marwa mohamed youss

Pathology Report

Clinical Data: Bilateral breast cancer, post NACT.
Nature Specimen: Bilateral breast conservative surgery.

1-Gross Examination of Right BCS:

• Procedure: Right BCS

Fibrofatty tissue piece measured 7x5x2.5 cm with covering skin ellipse measured 5x5 cm.
• Tumor extent: A wired and clipped greyish white tumor bed site.
• Size: 2.2x1.5x1 cm.

• Skin Involvement: show fixed nipple.

• Distance from margins: 0.2 cm away from (medial margin)

• Right extra medial margin: Fibrofatty tissue piece measured 5x4 cm free for gross induration.

• Right SLNs: Fibrofatty tissue piece 4x3 entangling 3 LNs up to 1x1 cm.

2-Gross Examination of Left BCS:

• Procedure: Left BCS

Fibrofatty tissue piece measured 8x6x2 cm with no skin cover.

• Tumor extent: Three wired greyish white tumor bed site.

• Size: 0.4x0.4, 0.5x0.5, 0.8x0.8 cm.

• Skin Involvement: show fixed nipple.

• Distance from margins: 1 cm away from (superior margin)

• Left extra deep: Fibrofatty tissue piece measured 4x3 cm free for gross induration.

• Left extra inferior: Fibrofatty tissue piece measured 4x2 cm free for gross induration.

• Left SLNs: Fibrofatty tissue piece 6x5 entangling 5 LNs up to 3.5x2 cm.

1-Microscopic Examination of Right BCS:

• Histological type: Residual viable invasive carcinoma. (Cellularity 10%)
• Ductal carcinoma in situ: No.
• Skin Involvement: Free.

Verified by:
Dr. Sagda El-Kady

Page 1 of 3

Printing By: Mrs. Mayada Hegazy

Name: زینب محمد مرسي MR. : 4699	
Detailed Medical Report	
السيد	
تحية طيبة وبعد,,	
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته	
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمتابعة وجد انه Ct4b , N2 Pyt came today for femara , supp tt for 3 mn DEXA scan 14-2-2026 >>> osteoprosis for dental consultaion for zometa CT CAP >>> R BSM 5-2025 >>> Free Patient needs extraction of lower right remaining roots. Zometa is postponed and dental panoramic xray is requested. 5/2026:ct CAP splenic focal lesion presented since 2023 (cystic lesion) fatty hepatomegaly uterine lesions likely fibrroids tday 18/5 /2026 for femara 2.5mg e supportive ttt then 3 months pharmacy. Femara +supportive*3m ct :splenic focal lesion for mri ,,,req next visit :28/9/2026. SM 15/8/2026 : NAD CT CAP 15/8/2026 : SD pt has splenic lesion 44x34mm (sd since 2023 no need for mri) for 3 month femara next visit 10/10.	
تحريراً في ,	
التاريخ :	19-09-2026
الوقت :	نهى سعيد علي محمد
الطبيب المعالج :	
يعتمد ,	



Printing Date : 01/07/2024 09:59:33

Page 2 of 3

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Room No. (4) - First Floor - Building No. 987, Nile Corniche - Sister El Qadima - Cairo, Egypt
Dr. Sagda El-Kady

Verified by:

Dr. Sagda El-Kady

RCB: 1.55, RCB-II
Stage: ypT2, ypNC,
Free SLNs (0/3).

Partial pathological response.
Free margins and free extra medial margin.
Residual invasive duct carcinoma.

Right BCS, post NACT.
1-Diagnosis of Right BCS:

- SLNs: Free (0/5)
- Fibrosis, histiocytic reaction and hyalinosis
- Additional pathological findings: Res of the breast show effect of therapy in the form of (
- Histologic response to therapy: Partial pathological response.
- Resection margins: Inferior margin showed extensive microcalcification, deep margin
- Ductal carcinoma in situ: 10% (solid and cribriform).
- Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
- Cellularity 70%
- Histological type: Residual multifocal invasive mixed ductal and lobular carcinoma.
- 2-Microscopic Examination of left BCS:
- Free SLNs (0/3).
- Surrounding breast tissue showing scattered papilloma.
- and hyalinosis.
- Additional pathological findings: Effect of therapy in the form of fibrosis, histiocytic reaction
- Microcalcifications: scattered microcalcification.
- Histologic response to therapy: Partial pathological response.
- Resection margins: Free margins & free extra medial margin.

Pathology Report

Request No. : 13006748
Request Date : 12/06/2024
Verification Date : 01/07/2024
Department : Out Patient
Financial Category : 12/06/2024
Registered by : Marwa Mohamed Youss

Referring Doc :
Age : 70 Y
Sex : Female
Pain No. : 2024/24
Patient No. : 126403240103429
Patient Name : Mrs. Mayada Hegazy

President's Initiative for Women's Health - Pathology Lab
Cairo, Egypt



Room No. (4) - First Floor-Building No. 987, NRC Corniche, NRC El Qadima - Cairo, Egypt

Printing By: Mrs. Mayada Hegazy

Page 3 of 3

Printing Date : 02/07/2024 09:59:33

Verified by :
Dr. Sagda El-Kady

Signature
Dr. Marwa Mahmoud
Dr. Sagda Elkady.

2- Diagnosis of left BCS:
Left BCS, post NAQT;
Residual multifocal invasive mixed ductal and lobular carcinoma, Grade II,
Free margins, free extra inferior margin and extra deep margins.
Free SLNs (0/5).
Stage: ypT1b, ypN0
- RCB: 1.843, RCB II class.

Pathology Report

Pathology Report	Referring Doc :	Age :	Sex :	Path No.	Patient No.	Patient Name
Registered by :	70 Y	Female	12024/24	126403240103429	126403240103429	126403240103429
Department :						
Financial Category :						
Verification Date :						
Request Date :						
Request No. :						
13006748						
12/06/2024						
01/07/2024						
Out Patient						
marwa mohamed youssa						

DR Marwa M. Hassan

lesion

-Right breast: (BIRADS 6), pathological proved retroareolar malignant lesion
-Left breast: (BIRADS 6), pathological proved two malignant lesions

Conclusion:

- ACR density b.
- Two ill defined UIC small lesion away from each other by 22mm
- Ultrasound demonstrated two speculated small dense lesion one between 12-1 o'clock measures 7x5mm other between 11-12 o'clock measures 7x8mm
- Normal skin/nipple/areolar complex.
- Enlarged one pathological lymph node measures 15x9mm with focal cortical thickening 5mm

Left breast

- ACR density b
- Retroareolar focal asymmetry with nipple reaction
- U/S shows ill-defined retroareolar hypoechoic area at zone (a&b) measures about 5x3cm
- Enlarged two axillary LNs 13x4mm & 14x6mm with focal cortical thickening in both 4.8mm

Right breast

High resolution ultrasound
Craniocaudally & Medio lateral oblique views

Technique

Detailed Findings

Clinical summary: pathological proved bilateral invasive duct carcinoma
Full field Bilateral Digital Mammography & ultrasonography

Name: zainab Mohamed morsy
Date: 12-11-2023
Women's Health Outreach Program



الوزارة العامة للصحة والسكان
Ministry of Health and Population

Presidential Program for Women's Health
Ministry of Health and Population



Dr Marwa M. Hassan

- This confirmed by mammography CC & ML lesion.
- clip marker was inserted in the retroareolar malignant proven mass
- spiculated malignant proven mass lesion, between 11-1 o'clock
- Three clips marker were inserted in the left multiple ill-defined irregular
- administered at the site of concern in the LT breast and right breast.
- Under complete sterile conditions, 3ml of local anaesthetic were

Technique:

receiving NACT.

Clinical summary: Known case of Bilateral invasive ductal carcinoma before

Imaging Guided Clip localization:

Name: Zainab Mohamed Morsy
Date: 27/08/2023

Ministry of Health and Population
Presidential Program for Women's Health



البرامج الصحية
Ministry of Health and Population





Ministry of Health
Kingdom of Saudi Arabia
Nasser Institute Hospital
Jeddah



Zainab Mohamed Moursy

09/05/2024

Dr. Zainab

Dynamic Contrast Enhanced MRI Study of the Breasts

Pulse Sequences:

Pre-contrast axial T1, T2, STIR and Sagittal T2WIs.

Post-contrast axial VIBE and Subtraction WIs.

Indication of the examination:

70 years old female patient complaining of both breast mass.

Request:

- Both breasts show mixed fibro glandular parenchyma (ACR B) with mild background enhancement.

- Status post clips insertion for histopathological proving left invasive duct carcinoma shows three residual mass lesions, at upper central (2 o'clock measuring about 5 mm), at upper outer (1 o'clock measuring about 5 mm), and at lower inner (12 o'clock measuring about 4 mm), exhibit intermediate signal on T1 WIs, low signal on T2 WIs, bright signal on fat sat images & restricted on DWIs, with heterogeneous contrast enhancement, on plotting time signal intensity curve, it shows type III

- Bilateral reactionary non-specific axillary lymph nodes.
- Right breast showing no residual masses could be detected.
- Normal nipple areola complex of both breasts.

OPINION:

- Status post clips insertion for histopathological proving left invasive duct carcinoma showing residual malignant looking breast mass as described above (BIRADS 6).

DR. ZAINAB NAJIB, M.D.

Women Imaging Consultant

Dr. Zainab Mohamed Moursy
Ministry of Health
Kingdom of Saudi Arabia
Nasser Institute Hospital
Jeddah

18/07/2024, 14:31

code 42 -- Name: A MDT form Hospital -- Name: E MDT form Hospital



Ministry of Health & Family Welfare
Government of India

Ministry of Health & Family Welfare
Government of India



MDT form

MDT form

MDT form

Patient Age: 20 Y

2-30 number: 2540240103428

A-Form 1

Date (DD/MM/YYYY)

Sex:

Form 1

2-Modified number

Old MDT

Date

30/08/2023

Reason of MDT

Date of MDT

09/07/2024

18/07/2024 D

Co morbidity

Present status

Past history

Risk factor

Family history

Social history

Personal history

Physical examination

Investigations

Management

Follow up

Signature

Date

Place

Signature

Date

Place

Clinical data

RT Breast retroareolar 5 cm lesion (clipped), axillary LN
Left Breast 3 masses (clipped), path axilla

RT Breast retroareolar 5 cm lesion (clipped), axillary LN
Left Breast 3 masses (clipped), path axilla

RT Breast retroareolar 5 cm lesion (clipped), axillary LN
Left Breast 3 masses (clipped), path axilla

RT Breast retroareolar 5 cm lesion (clipped), axillary LN
Left Breast 3 masses (clipped), path axilla

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Left Breast 3 masses (clipped), path axilla

RT Breast retroareolar 5 cm lesion (clipped), axillary LN
Left Breast 3 masses (clipped), path axilla

RT Breast retroareolar 5 cm lesion (clipped), axillary LN
Left Breast 3 masses (clipped), path axilla

MEDICAL ONCOLOGY

SURGERY

PSYCHIATRIC

SIGNATURE

RADIOLOGY

PATHOLOGY

NUTRITION

1-ADJUVANT AROMATASE INHIBITOR

MDT OUTCOME & PLAN

pt received AI for 4 months
NEW MRI BREAST REACTIVE AXILARY LNS
RT BCS residual IDC YPT2 (2cm), NO/3
IT BCS residual IDC & lobular
*YPT1B(MULTIFOCAL) YPN0/5

PATIENT HISTORY
RT BREAST RETROAREOLAR 5 CM LESION (CLIPPED), AXILARY LN
LEFT BREAST 3 MASSES (CLIPPED), PATH AXILLA

Sonography

Right T1

Right N1

Left N1

H

O

BR

7/8

7/8

Hx 2 Neg

GISH

K167

CT CAP & contrast free

Mole Workup 1

RT Breast retroareolar 5 cm lesion (clipped), axillary LN

Left Breast 3 masses (clipped), path axilla

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
SIGNATURE					

MDT OUTCOME & PLAN

Post-operative pathology:
 Rt ACS, residual IDC, free margin, free
 extra margin, free SLN 0/3, ypT2, ypN0
 Vt ACS, residual IDC & lobular, free margin,
 free extra SLN 0/5
 ypT1b, ypN0

Recommendations:

Mets Workup:

Sonomammography:

ERI: +ve
 PFI: +ve
 HER2: -ve
 Ki67: 10%

BX: LOCL

Clinical Data

Name	Dr. H. M. C. C.
Age	70yrs
Menopausal status	
NRS SCORE	
BMI	
Comorbidity	DM, HTN, Cardiac
MRN	3928/23
ID	
Phone	01126260695
Marital status	
Family history	Sister breast
REASON OF MDT	Sister breast

MDT FORM / 1 / 20 / G B

අයුතු ලෙස භාවිතයෙන් තොරව
 අනුමැතිය ලබා දෙනු ලබයි



Patient Name: Zainab Mohamed Morsi Hassan	
Exam: ultrasound BREAST (BOTH SIDE)	Medical ID: 3928/2023
Date: 09/08/2025	Pat. ID: 43227
Age: 71 Year(s)	

BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indications: Follow up after bilateral conservative surgery

- Breast density: Scattered deposits of glandular tissue, matching ACR B.
- Bilateral thickened breast skin with operative bed related focal asymmetry.
- No detected definite suspicious masses, architectural distortion or pathological calcifications in both breasts.

By comparison, bilaterally breast ultrasound:

- Heterogeneous (predominantly fatty) parenchymal echo-texture.
- Bilateral thickened breast skin (4 mm) with underlying minimal interstitial edema.

- No detected suspicious solid or cystic masses in both breasts.
- No detected significant axillary lymph nodes.

CONCLUSION:

• Bilateral post conservative changes (BIRADS 2 category).

Much Oblied

DR. MONA ZAKY MD

Professor of diagnostic radiology- Breast Imaging unit - Mansoura University

Signature Dr. Mona Zaki

09/08/2025 4:17 PM